

Tue, Dec 06, 2022 9:19AM • 32:29

Interviewer 00:00

Oh, no. All right. So [Name] Emotionally Focused Therapy. But you've also been trained with the EFT with emotion focused therapy. How long have you been working as a couple therapists now?

Therapist 00:15

Since [Year], nine, I met [Name] in [Year].

Interviewer 00:24

And have you always been working in [Country]?

Therapist 00:27

As a? As a therapist I always worked in Yes, I've only worked in [Country], and in [Language 1] and in [Language 1] and in [Language 2].

Interviewer 00:37

All right. So you primarily provide couples therapy to couples and friends.

Therapist 00:46

I provide I provide individual and couple therapy, I do a lot of training. So I'm a trainer. I'm an EFT trainer. And I trained a lot of therapists, but the couples therapy that I do, which are mainly interested in all that, that comes up in individual therapy, too. It's very, very relational individual therapy. And anyway, the couples therapy that I do is mainly for couples in [Country], but I do have had I worked with couples who have lived elsewhere as well.

Interviewer 01:17

But the training of therapists are global.

Therapist 01:21

The training I do with therapists, it's in [Language 1]. And so it's mainly people from [Country].

Interviewer 01:28

And are there specific types of couples that you provide your service at school do specialize in the profile of clients?

Therapist 01:39

I never really went in that direction. Now. I'm a generalist there. I see a lot of multicultural couples, multilingual couples. But I never really specialized in any kind of specific type of couple.

Interviewer 01:59

All right, and when your clients come to you, I know there's bound to be a broad range of issues, but what usually is the presenting issue or their even if they seek your service?

Therapist 02:11

Communication,

Interviewer 02:13

Communication, but—

Therapist 02:15

so there what we say what, what we hear EFT therapist, when we hear communication, is that we hear that there's not enough safety in the relationship, there's not a deep enough connection, and there's not enough safety. Because we know that when people aren't, don't feel the deep connection, deep emotional connection and the deep safety that that kind of connection brings, then they get, they have they get stuck in defensive reactions that create a negative dynamic, that increase the defensive reactions that perpetuates a negative dynamic. And you kind of go around in circles like that. So communication is we don't teach communication skills we teach people to become more connected when that's possible, and safe with each other.

Interviewer 03:12

All right, and do you do a formal assessment?

Therapist 03:18

I don't do I don't use scales or tests or questionnaires. No, I use I do an informal or I clinical assessment about whether they're good candidates for couples therapy. And that can last a few sessions, actually. So normally, there's a there's an initial appointment, where I meet them both I kind of hear their story. I asked them, what their what their what they want to achieve. So what do they want to be experiencing? When they finish couples therapy, what is it? So therapeutic goals, so to speak, but I don't like that word, because it's a it's very cognitive. And I really want to get them into a more experiential place. So it'll be more like if it's in [Language 2], it's more like, what's your vision for your couple? What do you want to be feeling in your relationship that you're not feeling now?

Interviewer 04:22

So initial appointment with them both. But he said that it might take a few sessions, what are the other sessions for?

Therapist 04:30

So also in the the initial session session, I'm going to see if they had the same agenda. So that's a that's when we work on the vision. I'm going to ask them if they have, I'm going to see whether they have the same agenda because sometimes there's one person that wants you know, that's in and the other person isn't really and so I assess that. And then usually what will happen is that if they're if the person who's not really motivated who's just kind of coming along to make the other person happy. I'll try to assess whether there is sufficient engagement to start couples therapy. And usually that can, the solo session will help. So usually what happens, the way I organize or the way we organize our sessions in EFT is there's a couple session where you assess their strengths, you kind of get to the get there a little bit of history, you kind of get an idea for how their current dynamic is. And you, you look at what their vision is for their couple, you know, what they want to be experiencing? Then there's a solo

session with each one of them. Where I come back to the there, there are a few things that happened in the solo session. One is that first thing is to talk about how they learned to love family of origin, attachment history. I also check, check in with, you know, what they said during their session for the vision? Is that still the case when the other person is not with them? Right? Are they were they being honest? And so and especially when they're diverging agendas, it's good to be able to assess whether, you know, they're sufficiently on the same page, enough to work together.

Interviewer 06:29

Okay, and then wanted to ask you as well, you said that you didn't use you know, pen and paper assessment scales? Was that always the case ever since the start of your practice? Or did you play around with some of these skills before?

Therapist 06:42

No

Interviewer 06:45

Why not?

Therapist 06:52

I think it's, I didn't really learn that way. I'm not I'm a humanist therapist. And so I didn't really learn that way. Or, I mean, I mean, I meet a person, right? I mean, and work with a person and so I'm not against it in any way. It's just hasn't been part of my practice.

Interviewer 07:16

All right. And currency year is different therapists and different researchers have characterized the construct of relationship quality quite differently from each other. I wanted to get your view about it for you what is the relationship quality?

Therapist 07:33

Yeah, I read your I read your article it's that's a pretty big question. For an EFT therapist, it has to do with intimate emotional connection. But a depth of connection, your high quality relationship implies of emotional connection, and safety. And in depth of emotional connection, I'm also including things like desire interest for the other person admiration, sexual desire, you know, so it's a there's, there's all of that in there.

Interviewer 08:21

So alright, so for you what differentiates couples with high relationship quality? And with those low relationship quality is this depth of connection? Would you say that like for couples with high quality relationship, there's more depth, more desire, more interest, more admiration, or sexual desire, and so on. And safety aAnd this might be another big question again, but for you like what makes other people what makes couples have these depth of connection and safety? Why isn't it you know, why do other couples don't have that? Same, the depth and safety.

Therapist 09:04

Um, people become attracted to each other? For different reasons, I think. But there's, there's always something when people are drawn to each other, there's always something going on. Even if it comes from a place of wounding from the past, right? Even if it comes from our wounded places, there's still attraction happening. And I think people need to be you know, it's important for people to be clear on what attracts them to the other person. But when the attraction is there, and it brings satisfaction, it's worth fighting for. But when people are really important to each other sometimes you know you're when the stakes are higher, right? You really act as a someone for whatever reason, and it can be Before from a place of wounding, right? This isn't this is my view. In any case, when you become very important to to another person, or when you're, when you're important to each other, the stakes are high. And when the stakes are high, you can hurt each other more easily. And when you hurt each other, then you can fall into these negative dynamics, which makes the hurt even greater. It's like you so you say that you hurt your partner without, without meaning to, by saying something not nice about their mother or something, you know, and then that's that can be it, it can be it can appear trivial to some people, but it might hurt deeply the person who is on the receiving end, because the person who said that is her refute her is her, their haven her safe havens. And so then she might develop, she or he might develop a defensive reaction, and kind of pull back or become or complain. And depending on the type of person, and then that makes that but that the other person feels it. And, and feels, for example, somebody's moving away slightly. And then that can make the other person react defensively, and become more kind of pursuing or, or critical or, more critical he becomes the more she withdraws and the more she withdraws to protect herself and to protect the relationship, the more he feels alone and scared and starts to pursue and criticize and it's actually often in the others is, it's actually often more often the man in sexual couples is the male partner who will withdraw and the female partner who will be more pursuing but not always, sexual couples, it's not you know, it's it can be it's is, it can at the same same patterns pattern can be there. Clearly, I'm clearly a more withdrawn person. And I'm, so I'm the female partner, but it's selling clearly and more withdrawn person in my life.

Interviewer 12:13

All right. And in your professional experience as well, like, what do you think are the most important things good to cultivate in a high quality relationship in a relationship that has safety, and you know, emotional depth, depth of connection, like what are the most important things in our relationship, to be able to achieve that

Therapist 12:38

is so you you said emotional depth and safety. So that's, that's what makes the relationship that's what creates a relationship quality. So that's what we work on. And it's being able to feel safe enough to look inside of yourself, and understand yourself and how you are reacting in the relationship. And then to feel safe enough to be able to, in the presence of the therapist express that to the other person. Now I feel I feel so alone in this relationship all the time. Or I feel so I wish it was all new. And I feel so worthless in this relationship all the time. And I don't want to be feeling like that anymore. And I feel like that because that's you know, and often people will be what they're experiencing in the relationship is also really just to their wounds.

Interviewer 13:39

Sorry, this is just a side question, curiosity. What happens when that for example, when another person says, Oh, now I feel because what have you because of what he said like because of what you shared because of your feelings. Now, I feel worse for making, you know, like you feel abandoned. Now I feel abandoned because you said you feel abandoned? How do you deal with cases like that, where they react to that sharing of vulnerability or hurt or wound?

Therapist 14:10

Well, it doesn't just in therapy, it doesn't just like come come out integrates. There's a whole build up to it. First of all, in EFT therapy, first of all the what people need to understand is their dynamic is how they've started there's kind of a very cognitive, it's not totally cognitive, but there's a kind of a cognitive phase where they they see their dynamic as the enemy. And they start to they start to understand that what the the the emotions that are on the surface of the secondary emotion is not is is not and what they're showing to their partner. So a defensive reaction and a secondary emotion is not the underlying issue. The underlying issue is that they're they're getting hurt and scared on an attachment level. So their primary attach about motions that are being triggered in the negative dynamic, the first phase of therapy is, is to define and help them understand their, their negative dynamic and help them understand the primary emotions, the attachment fears that are underneath. Then in the second phase of therapy, that's where, until they have understood their their dynamic, you can't go deeper. Because exactly what you describe will happen. Like, oh, well, they're not they're just not ready to, for themselves or to hear to hear themselves or to hear the other person.

Interviewer 15:39

Interesting. All right. And because this is a really like, particular way of viewing relationships are actually really informed by EFT about the safety about the depth of connection. I was wondering as well, if how you conceptualize your relationship, does it often differ from how your clients conceptualize the relationship? And did this impact the therapeutic process?

Therapist 16:07

Um, there's a, there's a kind of didactic side to EFT. And what you're saying what you're asking me is, sorry, I move up and down here? No. You're you're saying do couples see their relationship the same way? I do? Is that what you're saying? Is that what you're asking?

Interviewer 16:26

Yeah..So like, how you conceptualize their relationship and how they conceptualize relationships in general? Does that? Like, I'm pretty like, if there are differences that, you know, does that impact how you do your therapy?

Therapist 16:38

It depends. It depends, you know, sometimes a lot of people are, have read the book hold me tight. And they come into the therapy with an idea of, you know, they're attracted by this, this conceptual framework, you know, it's like it's an attachment. It's an attachment problem, right? So sometimes that happens. At other times there's like I said, there's a didactic side to EFT so we in therapy, they start to learn a certain way of looking at a relationship as an attachment as an attachment dilemma or

separation anxiety. The negative dynamic is created by separation anxiety. And so they learn it and there's a little you know, there's a little psycho Ed there going on all the time,

Interviewer 17:38

all right. But I wanted to ask as well the relationship is a complex system as we know it involves love and learn for sex relationship maintenance involves identity conflicts and so on. During therapy, how do you decide which one to focus on or which one in this complex system to target?

Therapist 18:01

First, you when we start therapy, we ask the people the couple, what the main themes of discord are. And then we kind of we look at how those themes interact with their dynamic you know how they are a part of their dynamic the themes will be part of their dynamic dynamic is created by lack of safety. So we're always going to be working on safety and connection safety and connection and the themes of discord like sexuality in laws money countries languages, you know, all these things, it's, you know, what language should we speak at home? I see a lot of so I see a lot of multicultural couples and that's you know, what language do we speak? Where do we spend Christmas all this stuff it's that all is when they're they become part of the negative cycle. And so it's more about creating safety and connection then resolving the issues do you understand what I'm saying is once you're safe with your partner, you can face the issues together and find solutions when you're not safe. You just you know the in laws, children languages, all will become part of the sex all will become part of the the negative cycle.

Interviewer 19:36

Yeah, interesting. And for how do you determine when to end couple therapy?

Therapist 19:41

So EFT therapy is a very structured therapy there are three different phases and you know, executive so there's there's stage one, which is about assessment, and working on the negative dynamic, helping them see their negative dynamic and the emotions that are the underpinnings of it. Stage two is restructuring the bond. So we go deep in stage two, it's of more interest psychic phase of therapy where people are going deep inside themselves and sharing with the other person to create that emotional connection and the safety. So it's like changing this, where you change the music and you change the dance, right? Sue Johnson says, and then stage three is consolidation. So the therapist is like a therapist is like walking through therapy with a map, you know exactly where you are all the time. So once they've finished, once they've done stage two, and main therapy events, and stage two are the main the therapeutic events that have been measured, in terms of determining efficacy, are the determining the negative dynamic: stage one.. Stage two, the more withdrawn partner really engages. But the main therapeutic event is the more pursuing partner who softens. If that is not achieved, that one, therapy is not finished. So it's, that's the hardest one to achieve.

Interviewer 21:22

All right. And from I know, again, there's gonna be a broad range, but from start to finish, usually how long how many sessions that they take.

Therapist 21:34

So [Name] does it in 12 sessions, but most of us do, like, you know, between 20 and 30.

Interviewer 21:41

And how many times do you guys meet, for example, every once every two weeks, once every week?

Therapist 21:50

With the kind of clientele I have, it's more an hour and a half every two weeks? I don't have I have a client and like a busy kind of travel and that kind of thing.

Interviewer 22:04

All right. All right. Okay, so as you know, from the email, we are currently developing a relationship quality scale that hopefully better represents relationship as a complex system that would hopefully help therapists and treatment planning and evaluation. I might Yes, your answer. But they need to ask anyway, how interested would you be in this kind of scale?

Therapist 22:28

Um, I would be really curious because it would, it would it would challenge kind of the idea is that we go into therapy with the EFT model. So I would be really interested, I'm very open. Like, I'm not like a purist, I guess it would be interesting for me to challenge kind of the the EFT model, a variant, or we'd be very interested, go be curious about it from from a more kind of clinical research kind of place. It would also be interesting on a more therapeutic level, like Okay, so what does this say about this? Couple of No, that would be that would be I would be interested? Definitely.

Interviewer 23:25

Interesting. Oh, because we, of course, we're currently developing this scale. How do we make sure that this scale would be useful in your practice? Or what would you be looking for in a newly developed scale like this, if you had like a wish list?

Therapist 23:42

Something that would help couples think about their relationship without getting freaked out about it. Without getting you know, without it bringing up like, creating a fight or that get them thinking, you know, okay. something constructive for the couples,

[Portion Redacted]

Interviewer 25:15

All right. And I need to ask again, you've already provided your support for this project by this interview. But are you still interested to collaborate on the development of the scale? But what I'm asking you that, as we proceed with this project, would I be Can I email you again, for example, if we are looking for more therapists and friends or data collection, their friends, etc.

Therapist 25:44

Sure.

Interviewer 25:45

Perfect. All right. Now, just before we end it, just like to recap everything that we've been saying, very quickly. In the course of this session, first we started out with me wanting to get a better sense of your clinical practice, you're primarily provide Emotionnally Focus Therapy but you've also been changed in EFT you started since [Year 1] [Year 2], which is when you met [Name], you primarily provide individual couple therapy in [Country], as well as training. And you're generally with your couples, the usual presenting issues for the couples are 'communication', but it's actually lack of safety and not enough connection. So you don't teach communication skills, you really provide that experiential feeling of safety and depth of connection. So you don't use any pen and paper skill that scales. What you do is a clinical assessment over the course of few sessions, getting their agenda, getting solid session, how they learn to love attachment, style, family history, whether they agree with a conjoined version of events. It's not that you are against scales, it's not how you learn, you know, it's not how you were trained, or you're more of a humanist as well, and you really meet with a person, but you're not against the practice of the pen and paper assessment. And so for you, when asked about relationship quality, it's really the intimate emotional connection and the depth of connection, and safety. And then this depth of connection that's also included there, that desire, the entrance admiration, the sexual desire, all of that gets in there. And we were also talking about other things connected to that like, even though, like when the stakes are high, you can more easily hurt each other. Again, you can Yeah. And that even if it comes from a place of wounding it should be it's important that couples are clear about why they're attracted to one another. So for you, again, nearly what differentiates of higher relationship quality and low relationship quality of this feeling of safety is this feeling of connection. And we are also talking about the sort of different conceptualization of relationships between you and the clients, it's really more of a them learning about, you know, viewing their relationship through the lens of EFT through the lens of an attachment, separation, anxiety, etc. So that's also really how you were replying, when it comes to the relationship as a complex system, there might be, you know, topics of sex topics of money, topics of like which languages you're seeking. But it's really as a boil down to it's really an issue of the process itself of the safety and connection, most of all, and when we talked about how to determine the end of couples therapy, we're talking about stages. stage one, stage two and stage three. Sorry, and just wanted to get a quick because he was talking about sort of the goal is that each stage is stage one, they should be able to understand the negative dynamics. Stage two, for example, the pursuing partners offense withdrawn partner engages more for stage three, what are the sort of like indicators that they're already down with this stage?

Therapist 29:12

It's a consolidation phase. So there were helping them supporting them and finding new solutions to old problems. And so when when they're stronger together, then the new solutions can.

Interviewer 29:30

Alright, so at stage two is basically the safety has been set. The connection has been set. To stage three islike, what do you do now that you're connected, and you're consolidating

Therapist 29:39

all that you're helping them consolidate the safety that they've been connection that they've achieved in their relationship?

Interviewer 29:46

Perfect. Alright, so for [Name], that's usually 12 weeks for you more 20 to 30 sessions, every two weeks, 1.5 hours so and we were talking about the development of the scale. For you, again, you're curious about that. It's not something that you would necessarily invest, you know, like monetary amount with. But if you had a wish list wish list for a scale like this, it would be something that would help couples think about their relationship without getting freaked out. So something constructive for them to reflect on their relationship. All right. Would you like to add anything to that?

Therapist 32:28

No, that was great.

Interviewer 32:29

All right. Now, before we end, I just wanted to talk to you about how we will handle the data. Of course, with you know, the shift in science now we want to be as transparent and as open as we are able to ethically and this project. So I wanted to ask which data sharing option you are most comfortable with. One option is that transcript portions of this interview can be shared, none of which can be identified to you. So if you can imagine it being written in a manuscript will just be like paragraphs, anonymous paragraphs, with upper relationship quality than anonymous paragraphs. Another option, if you were bolder, is that the whole transcript can be shared, removed of sensitive information and overly identifying information. So it will be anonymized. And readers can have a more better context of what we were talking about. Although in this option, there is a minimal chance that someone who knows you professionally, can piece together the details in the transcript and narrow it down among their colleagues about who they believe it is. So again, no sensitive information will be shared. But it will be anonymized. But there is a chance that you might get identified. So of these two data sharing options, which one are you more comfortable with?

Therapist 31:43

I'm perfectly fine with the last one.

Interviewer 31:47

All right. All right. So I will be sending you your transcript, one of these ones. Not very fast. If you want anything redacted, I redact it. All right. It has been really great. That has been you're having to session. Thank you so much.

Therapist 32:06

Thank you. Good luck,

Interviewer 32:08

and we will be having, I believe another social connection event next year. Will you be there again?

Therapist 32:14

I don't know. I don't know if I see if [Name] reaches out to me. I'm happy to make sure he does. Take care.

32:27

Bye.